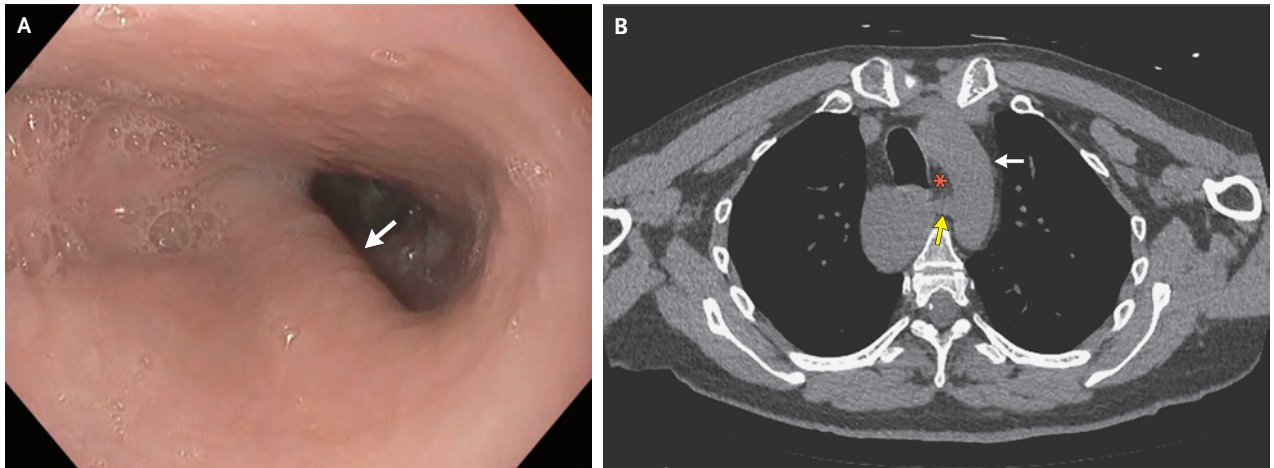


IMAGES IN CLINICAL MEDICINE

Dysphagia Lusoria



A 44-YEAR-OLD MAN WITH CEREBRAL PALSY WAS REFERRED TO THE Gastroenterology clinic for a 4-year history of choking episodes while eating. Despite treatment for reflux with a proton-pump inhibitor and food-texture modifications to address mild oropharyngeal dysphagia that had been identified in a modified barium-swallow study, the patient's symptoms had persisted. Physical examination was notable only for long-standing spastic quadriplegia and language delay related to his cerebral palsy. Endoscopic examination revealed luminal narrowing of the upper esophagus by a pulsating, cordlike structure in the posterior wall of the esophagus (Panel A, white arrow, and video). A subsequent computed tomographic scan of the chest showed an aberrant right subclavian artery (Panel B, yellow arrow) originating as a fourth branch of the aortic arch (Panel B, white arrow) and coursing posterior to the esophagus (Panel B, asterisk). A diagnosis of dysphagia lusoria — compression of the esophagus by an aberrant right subclavian artery — was made. Although dysphagia lusoria results from a congenital abnormality, it may not cause symptoms until middle age, when vascular stiffening occurs. Nonsurgical management with continued speech–language therapy for swallowing and dietary modifications was selected by the patient's health care proxy. Three months after diagnosis, the patient had less frequent choking episodes, continued to eat orally, and maintained his weight.

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A video is
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